

Derbyshire Shared Care Pathology Guidelines

Thrombocytopenia

Purpose of Guideline

GP referral guidance and further investigation of patients with thrombocytopenia

Definition

Platelet normal range is $150 - 450 \times 10^9/L$

Thrombocytopenia is defined as a platelet count $<150 \times 10^9/L$



The laboratory usually telephones platelet results $<30 \times 10^9/L$ to GP practices (when open) or to Derbyshire Health United when the GP practice is closed, unless the patient is a known Haematology patient.

Symptoms

Most patients with counts of $>50 \times 10^9/L$ are asymptomatic, with the risk of spontaneous haemorrhage increasing significantly below $20 \times 10^9/L$.

What are the main causes of thrombocytopenia?

- Alcohol, liver disease
- B12, folate deficiency
- Drugs (please review patient drug history and refer to SPC for each drug)
- Sepsis
- Hypersplenism (including due to portal hypertension)
- Disseminated intravascular coagulation (DIC)
- Immune consumption (ITP)
- Any cause of bone marrow failure (aplasia, malignant infiltration, myelodysplasia)
- Thrombotic Thrombocytopenic Purpura / Haemolytic Uraemic Syndrome

The following should be referred urgently for outpatient assessment:

- Platelet count $<50 \times 10^9/L$
- Platelet count $50 - 100 \times 10^9/L$ in association with:
 - other cytopenia (Hb $< 100 \text{ g/L}$, Neutrophils $<1 \times 10^9/L$)
 - splenomegaly
 - lymphadenopathy
 - pregnancy
 - upcoming surgery

Patients with platelets $<20 \times 10^9/L$ or active bleeding should be discussed with the duty / on-call Haematologist to arrange appropriate direct assessment

Appropriate investigation in primary care for patients not meeting criteria for urgent referral:

- Blood film examination – to exclude platelet clumping artefact (a blood film will be automatically added by the laboratory where there is significant thrombocytopenia, i.e. if platelets are <100 for the first time)
- Coag screen
- B12 and folate levels
- Alcohol history, other risk factors for liver disease including NASH
- Consider discontinuation of potentially precipitating medications
- Repeat FBC in 4-6 weeks

Referral for specialist opinion should be considered for:

- Persistent (at least on two occasions 4 - 6 weeks apart, no clumping noted on the blood film), unexplained thrombocytopenia $<100 \times 10^9/L$
- Where liver disease is suspected, any request for advice/ referral should be to Gastroenterology team

Contacts

Urgent queries:

RDH Consultant Haematologist

CRH Consultant Haematologist

QHB Consultant Haematologist

Via RDH switchboard

Via Consultant Connect or on-call Consultant through CRH switchboard

Via QHB Switchboard

Non urgent queries:

RDH Consultant Haematologist

CRH Consultant Haematologist

QHB Consultant Haematologist

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Haematology secretaries 01283 593432 or extn 4025/4032 via QHB switchboard

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Dr Chris Millar, Dr P Blackwell, Mrs H Seddon	Jan 2015	31 st January 2017
Dr Chris Millar, Dr P Blackwell, Mrs H Seddon	Feb 2017	28 th February 2019
Dr R Faulkner, Dr K Lam, Dr E Welch, Dr I Amott, Dr P Blackwell, Mrs H Seddon	June 2020	30 th June 2022
Dr J Tam, Dr K Lam, Dr E Welch, Ms H Seddon	Oct 2022	31 st Oct 2024
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